

Menopause — Diagnosis & Management

Clinical diagnosis · symptom management (hormonal & nonhormonal) · MHT risk/benefit counseling. IM facilitator reference.

Definitions & Diagnosis

- **Perimenopause:** irregular menses (changed interval) + hormonal symptoms; begins ~4 yr before LMP.
- **Menopause:** 12 mo amenorrhea (retrospective); mean age ~51.
- **Clinical diagnosis** — no testing if age >45 with classic symptoms. FSH is a poor single marker (SWAN).
- **Test if <45:** hCG, TSH, prolactin ± FSH. **<40** = full amenorrhea work-up (primary ovarian insufficiency).

Symptom Management

Symptom	Management
Hot flashes (mild)	Lifestyle: avoid heat/EtOH/caffeine/cigarettes; fan, layers; weight loss, exercise
Hot flashes (mod–severe)	MHT most effective; nonhormonal: SSRI/SNRI (venlafaxine, escitalopram, paroxetine), gabapentin; NK antagonists (fezolinetant, elinzanetant)
Mood / sleep	As for hot flashes; SSRI preferred if mood predominates (menopause ~2.5× depression risk)
GSM (dryness, dyspareunia, recurrent UTIs)	Moisturizers/lubricants; vaginal estrogen most effective, minimal systemic absorption
Menstrual irregularity	MHT; IUD or combined OCP until menopause

Avoid **fluoxetine**, **sertraline** for hot flashes (no benefit).

Menopausal Hormone Therapy (MHT)

- **Indication:** moderate–severe hot flashes. **Candidate:** <60 yr OR <10 yr post-menopause (low absolute risk).
- **Add progesterone** for any patient with an intact uterus (prevents endometrial hyperplasia).
- **Transdermal** estrogen < oral for VTE/stroke risk → preferred for moderate ASCVD risk & migraine.
- **Duration** ~5–10 yr, re-evaluate yearly, stop by ~60. Rebound VMS common on stopping.
- **Contraindications:** breast cancer, prior VTE/stroke/CHD, active liver disease, unexplained bleeding, high-risk endometrial cancer.

WHI & Risk/Benefit (ages 50–59, 5 yr/1000 women)

WHI (2002): excess CHD/stroke/VTE/breast cancer — but **mean age 63**, overstating risk for younger symptomatic women.

Outcome	E only	E + P
Coronary heart dz	–5.5	+2.5
Breast cancer	–0.5	+2.5
Stroke (CVA)	+1.5	+3
Hip/osteoporotic fx	–5.5	–5.5

Estrogen-alone profile is more favorable than E+P. Average absolute risk is small (~3 extra/1000/5 yr).

Newer Context (cite before teaching)

- **2022 Menopause Society (NAMS):** HT benefits outweigh risks for healthy symptomatic women <60 / <10 yr post-menopause; most effective for VMS & GSM, prevents fracture.
- **Fezolinetant** (Veoza, NK3 antagonist, FDA 2023) — monitor LFTs. **Elinzanetant** (Lynkue, dual NK1/NK3, FDA 2025) — ~73% VMS reduction, improves sleep.

Prevention & Pitfalls

- Estrogen loss → ↑ atherosclerosis & osteoporosis: lifestyle ± statin by ASCVD risk; calcium, vit D, exercise, stop smoking, ↓ EtOH.
- **Heavy bleeding in perimenopause** → **rule out malignancy:** hCG, pelvic exam, pelvic US ± endometrial biopsy.
- Contraception still needed in perimenopause: IUD to age 55, OCPs to age 51.